ONE Clinical Wound Protocol

POSSIBLE F-TAG/QMS

FEDERAL TAGS (F-TAGS)

F 697 Pain management

- The facility must ensure that pain management is provided to residents who require such services, consistent with professional standards of practice, the comprehensive person-centered care plan, and the residents' goals and preferences.

F 686 Treatment/services to prevent pressure/heal PU's

The facility must ensure that:

- A resident receives care, consistent with professional standards of practice, to prevent pressure ulcers, and does not develop pressure ulcers unless the individual's clinical condition demonstrates that they were unavoidable
- A resident with pressure ulcers receives necessary treatment and services to promote healing, prevent infection, and prevent new ulcers from developing

THERAPIES

- PT
- OT
- ST
- RN
- LPN

PURPOSE OF THERAPY

- To prevent wound development and preserve highest level of skin integrity
- To expedite or optimize wound healing
- To facilitate a **OneClinical** wound approach at facilities for best outcome
- To facilitate best practices for wound assessment and documentation
- To facilitate programming with options for advanced wound healing approaches

GOAL EXAMPLES

PT/OT:

- Patient will decrease L heel wound length, width, and depth, by .5 cm, to demonstrate progression towards epithelialization by X date and to allow for SPV level gait with RW.
- Patient will decrease L palm wound pain at rest from 5 to 3 to demonstrate wound progression and to promote overall patient well-being by X date to allow for independence when eating with a fork.
- L distal tibial wound will have minimal serosanguinous drainage as a demonstration of wound healing by X date to allow for independent donning/doffing of sock.
- Pt will decrease the area of R sacral wound from current XXcm2 to XXcm2 within 2 weeks in order to increase ability to move in/out bed comfortably.
- Pt will increase viable tissue in R sacral wound bed from current 50% to 80% granulation tissue within 2 weeks in order to reduce risk of infection.
- Pt will decrease pain noted in R sacral wound from current 5/10 to 2/10 using NRS within 2 weeks in order to improve general mobility.

TOOLS & ASSESSMENTS

Use of standardized/evidence based testing, such as:

- **BWAT -** Bates-Jensen Wound Assessment tool
- **PUSH -** Pressure Ulcer Scale for Healing
- **ABI –** Ankle Brachial Index
- **Capillary Refill Time**
- **Rubor of Dependency** vascular test
- **Monofilament Semmes-Weinstein** 5.07 sensory testing
- Dorsalis pedis and posterior tibial **pulses**
- **Edema** circumferential measurements and severity

COMPREHENSIVE SKIN ASSESSMENT which should include:

- Assessing for skin integrity and areas of pressure
- Assessing for discoloration of the skin
- Variations in warmth or coolness upon palpation
- Variations in moisture
- Variations in firmness, mobility, and turgor
- Skin lesions
- Assess skin under medical devices/splints and document condition consistently
- Assess & document newly closed/healed wounds to monitor skin integrity of this delicate skin post healing. (freq. of assessment determined by facility)

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POSSIBLE F-TAG/QMS

F 684 Quality of Care

- Each resident must receive, and the facility must provide the necessary care and services to attain the highest practicable physical, mental, and psychosocial well-being, in accordance with the comprehensive assessment and plan of care: SKIN

Dignity:

- Control odor
- Bandage strike-through cannot be present when resident is in common areas

Quality Measures

- New/Worsened Pressure Ulcers
- Moderate/Severe Pain
- Pressure Ulcers

THERAPIES

PURPOSE OF THERAPY

GOAL EXAMPLES

- Pt will decrease undermining on R sacral wound at 12 o clock by current 2 cm to 1 cm within 1 week in order to reduce infection risk.
- Pt will decrease tunneling on R sacral wound at 3 o'clock by current 3 cm to 1 cm within 2 weeks.
- Pt will have 100% understanding and return demonstration of donning PRAFO off-loading positioning device for R heel to provide pressure relief and prevent further skin breakdown in 1 week.
- Pt will demonstrate ability to provide pressure relief to L ischial tuberosity wound in WC with min assist to allow for improved wound healing in 2 weeks
- Pt will engage in rolling in bed with mod assist in order to assist with pressure relief and ADLs in 2 weeks.
- Pt will engage in sit to stand with mod assist to improve ability to offload sacral wound and reduce risk for further skin breakdown in 2 weeks.
- Pt will tolerate sitting up in wheelchair x 1 hour with necessary w/c positioning devices without c/o pain and without skin breakdown in 2 weeks.
- Patient will demonstrate compliance wearing Darco shoe for heel offloading of L heel wound 3/3 times in 1 week to facilitate healing.

TOOLS & ASSESSMENTS

- Assess general joint ROM and contracture management
- Smoking assessment & cessation education
- Assess appropriate nutrition and hydration
- Assess patient positioning: i.e. bed/mattress, W/C modifications, cushion/seating system

Elements for documentation and assessment use the MEASURE acronym, as following:

- **M** - Proper wound **measurement** using clock method, and accurate description of wound location.
- **E** - Documentation of **exudate** amount/quality.
- **A** - Documentation of **appearance** of wound/peri-wound, including tissue type, and percentage.
- **S** - Documentation of **suffering**/pain with appropriate tools such as VAS or Wong-Baker Faces scale.
- **U** - Documentation of **undermining** and tunneling, including location.
- **R** - Documentation of wound bed **reassessment**, including prognosis for healing, barriers to progress, and effectiveness of interventions.
- **E** - Documentation to include description of wound **edges** as attached, unattached, rolled (epibole), etc.

PCC Reports

- Resident Response Analyzer (Clinical Assessments)
- Braden Scale (nursing completes on admission, x4 weeks, quarterly, significant change). It includes components of Mobility, Activity, Sensory Perception, Nutrition, Friction/Shear, and Moisture.
- Diagnosis Report

PCC Reports

- Analytics of QM's

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